

THE SOMATIC RESET SERIES

Chronic Pain & the Nervous System

A Mind-Body Reset Guide

Disclaimer

This book is intended for educational and informational purposes only. It is not a substitute for professional medical, psychological, or psychiatric advice, diagnosis, or treatment. Always consult a qualified healthcare provider before making changes to your health routine, especially if you have a pre-existing condition.

The practices described here are general wellness approaches. Results will vary between individuals. The author and publisher accept no liability for outcomes arising from use of this material.

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Pain Is a Message, Not a Measurement

Pain is not a faithful measurement of tissue damage. It's a signal produced by the brain — a protective output generated when the brain concludes that something requires attention. This distinction is not semantic. It changes everything about how chronic pain can be approached.

The New Understanding of Pain

For most of the twentieth century, pain was understood as a simple input-output system: damaged tissue sends a danger signal, the brain registers it as pain. The intensity of pain was assumed to correspond to the extent of damage.

This model has been comprehensively challenged by modern pain neuroscience. Lorimer Moseley and David Butler, among others, have demonstrated that pain is an output of the brain, not an input from the body — a protective response generated when the brain perceives threat, regardless of whether tissue damage is present or proportionate to the pain experienced.

This explains findings that previously seemed inexplicable: phantom limb pain (pain in a limb that no longer exists), the fact that many people with severe structural changes on MRI have no pain while others with minimal findings have disabling pain, and the well-documented phenomenon of placebo — pain that increases when patients are told something is wrong with them, even when nothing has changed structurally.

Central Sensitisation

In chronic pain, the nervous system itself becomes sensitised — the alarm system turns up its sensitivity, producing pain from inputs that would not normally register as painful and amplifying those that would. This is called central sensitisation, and it's the primary mechanism in conditions like fibromyalgia, chronic widespread pain, and many presentations of chronic back and neck pain.

Understanding central sensitisation changes the treatment target. The problem is not (only) in the tissues. It's in the sensitivity of the nervous system itself. And the nervous system responds to exactly the kinds of practices described throughout this series — with modifications for chronic pain contexts.

*"Pain is real. It is not imagined. But it is also not
an accurate GPS reading of where the
damage is or how bad it is."*

Calming the Sensitised System

A sensitised nervous system cannot be argued with. It can be gradually recalibrated.

Safety Signals for a Pain-Sensitised Nervous System

The brain increases pain output when it concludes that the body is in danger. Reducing pain, over time, involves increasing the brain's sense of safety — through information, experience, and physiological input. Education itself has been shown to reduce pain: understanding that chronic pain is a sensitisation phenomenon rather than evidence of ongoing damage reduces the threat value the brain assigns to the sensation, which reduces the pain output.

Breathwork, gentle movement, social connection, and positive sensory experiences all send safety signals to the pain-producing systems. They don't override the pain immediately. But over time, consistent safety inputs recalibrate the system's threat assessment.

Movement Despite Pain — The Graded Approach

Avoiding movement in chronic pain feels logical — moving hurts, so don't move. But the evidence is clear that movement avoidance strengthens the central sensitisation cycle. The nervous system interprets movement avoidance as confirmation that movement is dangerous, which maintains and often increases pain sensitivity.

Graded motor imagery and graded exposure to movement, developed by Moseley and others, involve gradually introducing movement in small, non-threatening increments — beginning with imagining movement before performing it, starting with very small ranges of motion, and incrementally expanding. The goal is to provide evidence to the brain that movement is safe.

Breathwork and Pain Perception

Slow, diaphragmatic breathing reliably reduces pain perception through multiple mechanisms: it activates the parasympathetic system, reduces cortisol and adrenaline (which amplify pain), and shifts attention. Research consistently shows that breathwork reduces both self-reported pain intensity and the neural signatures of pain on brain imaging.

Coherent breathing (five breaths per minute, as described in the second book) is particularly well-studied for pain conditions. Consistent daily practice over weeks to months produces lasting changes in pain sensitivity through its effects on HRV and vagal tone.

"The nervous system learns. It learned to amplify pain. With the right inputs, it can learn to turn down the volume."

The Emotional Dimension of Chronic Pain

Chronic pain and emotional pain share neural pathways. This is not a metaphor.

Adverse Experiences and Pain

The ACE (Adverse Childhood Experiences) study found a strong dose-response relationship between the number of adverse childhood experiences and the likelihood of chronic pain conditions in adulthood. Other research has replicated this finding across multiple pain conditions.

This is not saying that chronic pain is 'all in the head' or is imagined. It is saying that the nervous system that developed under adversity developed different threat-sensitivity calibrations, which over time express in pain responses. The biological pathway from early adversity to chronic pain runs through the nervous system, the HPA axis, and inflammatory pathways — all real, all measurable.

Trauma-Informed Approaches to Chronic Pain

Working with chronic pain from a nervous system perspective often means working with the emotional history that shaped the nervous system's sensitivity. This doesn't require reliving trauma or psychologising physical symptoms. It means creating enough nervous system safety to allow the system to recalibrate.

Somatic Experiencing, TRE, and EMDR have all shown promise in research contexts for chronic pain conditions, particularly where trauma or adverse experience is part of the history. A trauma-informed physiotherapist or pain psychologist trained in these approaches is worth seeking if chronic pain is your primary concern.

"The body has its reasons. Understanding them is not the same as agreeing that the pain is inevitable."

A Daily Practice for Chronic Pain

Chronic pain management through nervous system regulation is a long game. The practices are simple; the commitment is sustained.

The Core Daily Practice

For chronic pain specifically, the daily practice prioritises safety and consistency over intensity. Ten minutes of coherent breathing morning and evening. One brief period of gentle graded movement. One practice of deliberate engagement with positive sensation — not to override the pain, but to give the nervous system evidence that not everything is threatening.

The positive sensation practice is often overlooked. A warm shower attended to with full sensory attention. A cup of tea savoured. A brief period of being in a comfortable position and noticing what feels okay in the body, rather than scanning for what doesn't. These small interventions shift the data the brain is working with.

Pacing and Energy Management

Chronic pain is often accompanied by fatigue and a boom-bust activity pattern that amplifies both. Pacing — doing a consistent, moderate level of activity rather than alternating between overdoing and crashing — is the evidence-based approach to managing the energy economics of chronic pain.

The aim is to find the activity level that can be sustained consistently without producing flare-ups, then gradually and incrementally expand from there. This is slower than the instinctive approach (push through the good days, collapse on the bad ones) but produces more consistent function over time.

"Managing chronic pain is not about willpower over pain. It's about intelligence applied to the system that produces it."
